

D220

Information Technology in Nursing Practice

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A nurse on a telemetry unit with new wireless monitors records a patient's heart rate as 128 at 1400. The patient's daughter is a paramedic. The nurse lines this reading up with the prior three and sees a rising trend. What DIKW level has the nurse reached?

- A. Knowledge, because 128 is always abnormal
- B. Information
- C. Data
- D. Wisdom

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

At which DIKW level does a nurse use knowledge and experience to decide the best action for a particular patient?

- A. Knowledge
- B. Wisdom
- C. Information
- D. Data

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A community hospital with a new cafeteria sends a discharge summary to a rehab facility that uses a different EHR vendor. The rehab system reads the summary and automatically places the medication list into its own medication reconciliation screen. What does this demonstrate?

- A. Foundational interoperability only
- B. Usability
- C. Data mining
- D. Semantic interoperability

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A nurse on a busy 40-bed med-surg floor hears that a well-known local news anchor was admitted to the ICU. The nurse has a valid EHR login and has worked at the hospital eight years. Is it appropriate for her to open the anchor's chart?

- A. Yes, because she has a valid login.
- B. No; access is limited to patients she has a job-related need to view.
- C. Yes, as long as she does not share what she sees.
- D. Yes, because her eight years of service grant wider access.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

Which access to protected health information is appropriate?

- A. A nurse checks her neighbor's lab results out of concern.
- B. A nurse posts a wound photo with the room number visible.
- C. A case manager reviews an assigned patient's record to plan discharge.
- D. Any clinician may view any record in the facility.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

The EHR at a hospital that recently upgraded its servers fires a drug-interaction alert when a nurse scans a new medication order. She has seen many low-value alerts this shift. The pharmacist is at lunch. What is the best action?

- A. Read and evaluate the alert, hold the dose if warranted, and verify with the prescriber or pharmacy.
- B. Wait for the pharmacist to return before reading the alert.
- C. Follow every alert automatically without further thought.
- D. Override it, since most alerts today have been low value.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

A unit's alert override rate has climbed to 95%, and staff say most alerts are irrelevant. What problem does this signal, and what is the system-level fix?

- A.** Add more alerts to catch errors staff are missing.
- B.** Turn off all alerts.
- C.** Alert fatigue; tune alerts so they are specific and clinically meaningful.
- D.** Staff noncompliance; discipline everyone who overrides.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

On a night shift on a unit with a new scanning system, a patient's wristband will not scan. The patient is alert and has been on the unit five days. A coworker suggests scanning a spare wristband taped to the medication cart. What should the nurse do?

- A.** Never give any medication without a scan, even in an emergency.
- B.** Skip scanning, since the patient is alert and well known.
- C.** Scan the spare band so the record stays complete.
- D.** Decline the workaround, confirm identity with two identifiers, and get a replacement wristband per policy.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A health system with 12 hospitals wants to compare nursing outcomes such as pressure injury rates across sites. Each site has documented in free text. The merger that formed the system was covered by the local paper. What should it adopt?

- A.** Standardized terminologies such as SNOMED CT so the same concept is coded the same way at every site.
- B.** Larger free-text fields so nurses can write more.
- C.** A single EHR vendor, which by itself guarantees comparable data.
- D.** Faster servers at every site.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A hospital that just hired a new CNO plans to replace its nurse scheduling software. Before any vendor is chosen, a team interviews nurses about workflow problems and documents requirements. The budget is \$250,000. Which SDLC phase is this?

- A.** Design
- B.** Evaluation
- C.** Implementation
- D.** Planning and analysis

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

Six months after go-live, a team compares medication error rates before and after a new eMAR. Which SDLC phase is this?

- A.** Evaluation
- B.** Analysis
- C.** Implementation
- D.** Design

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A patient with diabetes who uses a fitness watch logs her glucose readings in an app she controls and chooses to share with her clinic. She also sees three specialists. What is the app?

- A.** An electronic health record (EHR), since any digital health data is an EHR
- B.** A clinical decision support system
- C.** A personal health record (PHR)
- D.** An electronic medical record (EMR)

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A nurse on a telemetry unit ~~with new wireless monitors~~ records a patient's heart rate as 128 at 1400. ~~The patient's daughter is a paramedic.~~ The nurse lines this reading up with the prior three and sees a rising trend. What DIKW level has the nurse reached?

Target word: DIKW level **Decides:** Raw readings organized into a trend = information.

A	Absolute language	No interpretation linking the trend to a cause has been made yet.
B	Correct	Data given context and organization, but not yet interpreted.
C	Confusion set	A single raw reading would be data; the trend adds context.
D	Adjacent category	Wisdom is choosing the best action for this patient.

2. Answer B Concept 1 • transfer

At which DIKW level does a nurse use knowledge and experience to decide the best action for a particular patient?

Target word: level from description **Decides:** Applying knowledge to act = wisdom.

A	Adjacent category	Knowledge identifies patterns but is not yet the decision.
B	Correct	Wisdom is the judicious use of knowledge to act.
C	Confusion set	Information is organized data without interpretation.
D	Inverted operation	Data is the lowest level, not the highest.

3. Answer D Concept 2

A community hospital ~~with a new cafeteria~~ sends a discharge summary to a rehab facility that uses a different EHR vendor. The rehab system reads the summary and automatically places the medication list into its own medication reconciliation screen. What does this demonstrate?

Target word: level of interoperability **Decides:** The receiving system interprets and uses the data.

A	Dropped element	Foundational is just transmission; here the data is used.
B	Adjacent category	Usability is ease of use for people, not data exchange.
C	Confusion set	Data mining finds patterns in large data sets.
D	Correct	Meaning is preserved so the data can be used, not just displayed.

4. Answer B Concept 3

A nurse on a ~~busy 40-bed~~ med-surg floor hears that a well-known local news anchor was admitted to the ICU. The nurse has a valid EHR login and ~~has worked at the hospital eight years.~~ Is it appropriate for her to open the anchor's chart?

Target word: appropriate access **Decides:** She has no job-related need for this patient's record.

A	Adjacent category	Authentication is not authorization.
B	Correct	Minimum necessary: access follows role and need.
C	Dropped element	Viewing without a need is already a violation.
D	Camouflage	Seniority does not create a need to know.

5. Answer C Concept 3 • transfer

Which access to protected health information is appropriate?

Target word: pick the permitted access **Decides:** Access tied to an assigned job duty.

A	Confusion set	Good intent is not a job-related need.
B	Dropped element	Identifiers remain, so it is not de-identified.
C	Correct	A job-related need for that patient.
D	Absolute language	Access is limited by role and need.

6. Answer A Concept 4

The EHR at a hospital ~~that recently upgraded its servers~~ fires a drug-interaction alert when a nurse scans a new medication order. She has seen many low-value alerts this shift. ~~The pharmacist is at lunch.~~ What is the best action?

Target word: response to a CDS alert **Decides:** Alerts support judgment; this one must be evaluated.

A	Correct	Uses the alert to inform clinical judgment.
B	Camouflage	The pharmacist's lunch does not decide what the nurse does first.
C	Absolute language	CDS supports judgment; it does not replace it.
D	Inverted operation	That is alert fatigue driving the decision.

7. Answer C Concept 4 • transfer

A unit's alert override rate has climbed to 95%, and staff say most alerts are irrelevant. What problem does this signal, and what is the system-level fix?

Target word: name the problem and fix **Decides:** Too many low-value alerts cause habitual overrides.

A	Inverted operation	More alerts worsen fatigue.
B	Absolute language	Removes useful safety checks along with the noise.
C	Correct	Fewer, better alerts restore attention.
D	Adjacent category	Blames people for a system design problem.

8. Answer D Concept 5

On a night shift on a unit ~~with a new scanning system~~, a patient's wristband will not scan. The patient is alert and ~~has been on the unit five days~~. A coworker suggests scanning a spare wristband taped to the medication cart. What should the nurse do?

Target word: safe BCMA practice **Decides:** A spare band breaks the link between scan and patient.

A	Absolute language	Policies allow verified downtime procedures in emergencies.
B	Camouflage	Length of stay does not replace verification.
C	Dropped element	Drops the actual patient verification.
D	Correct	Keeps the safety check tied to the real patient.

9. Answer A Concept 6

A health system with 12 hospitals wants to compare nursing outcomes such as pressure injury rates across sites. Each site has documented in free text. ~~The merger that formed the system was covered by the local paper.~~ What should it adopt?

Target word: what makes data comparable **Decides:** Concepts must be coded the same way everywhere.

A	Correct	Coded data can be aggregated and compared.
B	Inverted operation	More free text makes comparison harder.
C	Absolute language	Same vendor without shared coding can still produce inconsistent data.
D	Adjacent category	Speed does not make data comparable.

10. Answer D Concept 7

A hospital ~~that just hired a new CNO~~ plans to replace its nurse scheduling software. Before any vendor is chosen, a team interviews nurses about workflow problems and documents requirements. ~~The budget is \$250,000.~~ Which SDLC phase is this?

Target word: SDLC phase **Decides:** Gathering needs before selection = analysis.

A	Adjacent category	Design builds the solution from the requirements.
B	Inverted operation	Evaluation comes after go-live.
C	Confusion set	Implementation is build, test, and go-live.
D	Correct	Needs assessment and requirements come first.

11. Answer A Concept 7 • transfer

Six months after go-live, a team compares medication error rates before and after a new eMAR. Which SDLC phase is this?

Target word: SDLC phase **Decides:** Measuring results after go-live = evaluation.

A	Correct	Judges whether the system met its goals.
B	Inverted operation	Analysis happens before design.
C	Adjacent category	Implementation ended at go-live.
D	Confusion set	Design is before the system is built.

12. Answer C Concept 8

A patient with diabetes ~~who uses a fitness watch~~ logs her glucose readings in an app she controls and chooses to share with her clinic. ~~She also sees three specialists.~~ What is the app?

Target word: record type **Decides:** The patient manages and controls it.

A	Absolute language	EHRs are maintained by providers and shared across organizations.
B	Adjacent category	CDS gives clinicians guidance; it is not a record.
C	Correct	PHRs are managed by the patient.
D	Confusion set	An EMR is one provider organization's record.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	4	
Wrong stem number	A figure copied from the prompt that is not the answer.	0	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	6	
Absolute language	Always, never, guarantee — where judgment applies.	7	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	3	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Data, information, knowledge, wisdom (DIKW)	Data are raw facts; information is data given context; knowledge links information into patterns; wisdom applies knowledge to choose the right action.	Calling any organized number “knowledge.”	A single reading vs. a trend vs. an interpretation vs. a decision.
2	Interoperability	Semantic interoperability means a receiving system can interpret and use data from another system, not just receive it.	Thinking a successful file transfer equals interoperability.	Data from one vendor used automatically in another.
3	HIPAA minimum necessary	Access to PHI is limited to what a person needs for their job; a valid login is not permission.	Equating authentication with authorization.	Curiosity, a well-known patient, or a coworker’s record.
4	Clinical decision support and alert fatigue	CDS supports, not replaces, clinical judgment; too many low-value alerts cause overrides, fixed by tuning alerts at the system level.	Overriding by habit, or adding more alerts to fix overrides.	An alert firing and a high override rate.
5	Barcode medication administration	BCMA works only if every scan matches the real patient and drug; workarounds defeat the safety check.	Scanning a spare band or skipping the scan for a known patient.	A scan failure and a shortcut offered.
6	Standardized terminologies	Coded terminologies (e.g., SNOMED CT, LOINC) let the same concept be recorded the same way so data can be compared and shared.	Thinking a single vendor or more free text makes data comparable.	Comparing outcomes across sites.
7	System development life cycle	Planning/analysis (needs and requirements) → design → implementation → evaluation; nurses belong in every phase, especially analysis.	Confusing requirements gathering with design or evaluation.	What the team is doing and when (before vendor choice, after go-live).
8	EMR vs. EHR vs. PHR	An EMR is one organization’s record; an EHR is shareable across organizations; a PHR is managed by the patient.	Calling any digital health record an EHR.	Who controls the record.

Course scope (public description): Information Technology in Nursing Practice provides a basic overview of information technology as it relates to the baccalaureate-prepared nurse. It is a foundational overview of nursing informatics with an emphasis on developing basic competency. This course teaches students that nursing informatics synthesizes nursing science, information science, and computer science through health applications to support decision-making in a dynamic healthcare environment. All prior courses in the sequence for this program serve as prerequisites for this course.